

1.0 Executive Summary

Abbott is a four-segment diversified operator: Medical Devices, Diagnostics, Nutrition, and Established Pharmaceuticals, now with Exact Sciences acquisition bringing its oncology expertise into the Diagnostics segment. Medical Devices (~\$21B in revenue) leads the portfolio, anchored by diabetes care (specifically continuous glucose monitoring) and a broad cardiovascular device range. Diagnostics (~\$9B) supplies institutional lab equipment plus the reagent ecosystem that feeds it, while it is currently undergoing a demand normalisation following the subside of Covid-19 pandemic. Nutrition (~\$8.45B) runs the Ensure, Glucerna, and Similac brands. Established Pharmaceuticals (~\$5.5B) sells branded generics in emerging markets.

Abbott operates in over 160 countries and is currently led by Robert Ford from 2020. It experienced a strong growth during the Covid year, which was attributable to its highly accurate Covid self-test kit. However, it is experiencing some legal and normalisation headwinds that contribute to the ~36% drop in its share price. We are here to see if the condemning factors are true signals or they are just some short-term noise.

2.0 Business Segment Breakdown

Diagnostics

The Diagnostics segment is structurally misread by anyone who focuses on the hardware. Abbott places large laboratory instruments, immunoassay analyzers, PCR platforms, automation track systems, into hospitals and clinical labs, often at subsidized or near-cost pricing. It operates on a razor-and-blade model.

The actual margin engine is the proprietary reagent consumables that flow through those instruments indefinitely afterward. Hospitals that accept the hardware commit to minimum annual test volumes under multi-year contracts, typically three to five years. What appears to be a capital equipment sale is, in economic reality, a long-duration annuity.

The lock-in goes deeper than the contract. Abbott's Total Laboratory Automation (TLA), centered on the GLP Systems Track, is a strategic ecosystem lock-in whereby the laboratory's physical workflow and Alinity instruments are tethered to the GLP track. This physical lock-in provides Abbott the leverage to sell higher-margin reagents, which is reflected in Abbott's \$6.1

billion backlog as of late 2025. Once a hospital has been built around that system, the cost of switching reagent suppliers is no longer just contractual. It is architectural. The hospital would need to rebuild the physical layout of its lab.

While the demand for Covid diagnostic tests has normalised, Abbott aims to fill the gap with emerging oncology diagnostics needs by acquiring Exact Sciences. This move will add both noninvasive screening infrastructure (Colorguard) and profiling tools (Oncotype DX) to Abbott's diagnostic portfolio, in a light of insulating the company from the volatility of infectious disease cycles and catching the rising demand of cancer treatment worldwide.

Medical Devices

Medical Devices is the largest segment, representing roughly 45% of total group revenue. It spans six business units: Diabetes Care (CGM), Cardiovascular, Electrophysiology, Structural Heart, Rhythm Management, and Neuromodulation. Beneath those six labels sit three structurally distinct ways of making money.

The high-frequency consumable model governs Diabetes Care. FreeStyle Libre sensors are replaced every 10 to 14 days, producing roughly two dozen repurchase events per patient per year. The reader hardware is a subsidised entry point. The sensor is the margin engine. Abbott is applying the familiar razor-and-blade model here. The moat compounds through three reinforcing layers: patient habituation to the sensor interface and app, prescriber integration into the LibreView clinical data platform, and reimbursement pathway incumbency in established markets. Abbott's primary growth vector is not taking share from Dexcom in the insulin-dependent diabetic base. It is expanding into the Type 2 non-insulin-dependent population, a substantially larger cohort that has historically sat outside the Continuous Glucose Monitoring (CGM) market entirely. However, this segment is facing a structural risk where CGM is subjected to competitive bidding which is going to compress the pricing power of Abbott and other CGM suppliers. Suppliers are required to submit bids to provide the equipment at discounted prices, the competitive nature of it is going to drive down selling price. Nonetheless, this mechanism also draws criticism as the competitive pricing will squeeze out inefficient players, which is going to benefit the larger players as competition drops. Some opine that Abbott faces the least impact as the company's pricing strategy is already comparatively aggressive as compared to its peers.

The procedure-driven single-use model governs Electrophysiology and Cardiovascular. Volt PFA catheters and XIENCE coronary stents are consumed once per procedure at high average selling prices. Revenue scales with procedure volume rather than with an installed patient base, making these sub-segments more sensitive to hospital capital cycles and clinical adoption rates. Electrophysiology is the fastest-growing area within the segment, driven by pulsed field ablation, a cardiac ablation method that is faster and carries fewer complications than legacy thermal techniques. Abbott's first-mover position with the Volt catheter is the growth catalyst here, though Biosense Webster and Medtronic are advancing their own PFA platforms, which will narrow that window over time. XIENCE, by contrast, is a mature franchise that competes on the strength of a decade of clinical outcomes data and hospital formulary incumbency rather than technology leadership.

The implant-plus-periodic-replacement model governs Rhythm Management, Structural Heart, and Neuromodulation. Pacemakers, ICDs, MitraClip, and deep brain stimulation systems are high-ASP, one-time placements with replacement cycles of five to ten years depending on battery life. The periodic generator replacement creates a lower-frequency but structurally predictable recurring revenue stream. The deeper moat in this category is not the hardware but the remote monitoring and software ecosystems layered on top of it: Merlin.net for cardiac rhythm devices, NeuroSphere for DBS. These platforms accumulate years of patient data and become embedded in physician clinical workflows, making migration to a competitor's device meaningfully costly even when the implant reaches end-of-life. The switching cost is informational and habitual rather than mechanical.

Established Pharmaceuticals

EPD is the segment that attracts the least investor attention and operates on a logic that is frequently misclassified. This is not a pharmaceutical business in the conventional sense. There is no pipeline, no binary drug approval risk, and no dependence on patent exclusivity. Abbott sells branded generics, off-patent medicines where the formula is public knowledge, but the Abbott name and its reputation for quality consistency command a price premium above unbranded commodity substitutes. More importantly, no intensive R&D spending is needed to develop new drugs, where the success rate of entering clinical development to getting FDA approval is around 10%.

The commercial foundation is cognitive loyalty rather than legal monopoly. In emerging markets across Asia Pacific, Latin America, and Middle East & Africa, prescribing behaviour is shaped by institutional familiarity and trust in ways that do not apply in developed market pharmacy channels. A physician who has been prescribing Duphaston for fifteen years is not easily converted to a Viartis equivalent, even at a lower price, unless the differential exceeds the threshold of clinical confidence. Abbott's EPD brands: Creon, Duphaston, Duphalac, Brufen, hold that kind of entrenched prescribing history in their respective markets.

Nutrition

Nutrition generated approximately \$8.45 billion in revenue in 2025 and operates on a fundamentally different competitive architecture than a conventional consumer goods business. It runs across two sub-segments: Pediatric Nutrition (Similac, PediaSure) and Adult and Clinical Nutrition (Ensure, Glucerna, Nepro).

Abbott does not compete in this segment primarily through advertising spend or retail shelf positioning. It competes by embedding its brands into clinical recommendation pathways before the purchase decision is ever made. Ensure and Similac hold the number one doctor-recommended position in their respective categories. When a paediatrician recommends Similac to a new parent, or when a hospital dietitian includes Ensure in a patient's discharge care plan, the commercial outcome has already been determined at the institutional level. The parent and the patient are not making independent consumer choices, they are executing a clinical recommendation. Displacing that brand requires a competitor to convert the clinician, not the consumer, which is a structurally slower and more expensive process than winning a price promotion at retail.

The clinical moat is reinforced by two structural features. First, disease-specific formulas carrying FDA or EFSA-registered health claims like Glucerna for glycaemic management and Nepro for renal patients, command a meaningful price premium over unbranded equivalents. Replicating those regulatory claims requires equivalent clinical trial investment and clearance cycles, making them a genuine structural barrier rather than a marketing artefact. Second, Abbott holds exclusive WIC program contracts across 42 US states, supplying subsidised infant formula under contracts that typically run two to four years. Competitors cannot penetrate this volume floor through pricing alone, they must win a state tender process, which insulates Abbott's

pediatric volume base from normal market competition for extended periods. The WIC channel is not a high-margin business in isolation, but it provides volume certainty that underpins manufacturing scale and unit cost efficiency across the facility network.

However, this segment has recently encountered significant litigation headwinds regarding allegations that its Similac infant formula causes necrotizing enterocolitis (NEC) in premature babies. We detail the potential impact of these lawsuits in the following section.

3.0 Thesis

Abbott Laboratories' recent share price weakness has been driven by two headline risks:

1. NEC litigation
2. EPS dilution following the Exact Sciences acquisition

both of which, upon closer examination, represent either overstated fears or temporary transition costs rather than structural impairments to the business.

I. NEC Litigation:

The most visible overhang on Abbott's valuation is the wave of NEC (necrotizing enterocolitis) lawsuits. This is indeed a risk that can't be overlooked as there are over 1,700 pending cases shared between Abbott and Mead Johnson, alleging that premature infant formula failed to adequately warn of NEC risk. However, I think that despite the risk, it does not translate to any fundamental shift to Abbott's business quality. Why?

First and foremost, this is an industry-wide litigation phenomenon, not a company-specific failure. Abbott does not stand alone in the dock; the entire premature infant formula category is being challenged simultaneously. Attributing a systemic legal campaign to Abbott's product quality or corporate governance is a categorical error that the market has nonetheless made.

More critically, the scientific consensus does not support the plaintiff's central thesis. The medical community has been clear: there is no strong evidence that suggests the premature infant formula contains toxic elements. What the research actually shows is that it is predominantly the absence of human milk, not the presence of formula, that is associated with elevated NEC risk. Stated plainly, the risk lies in what is missing, not in what is being administered. Further

reinforcing this, research confirms that human milk significantly reduces but does not eliminate the risk of NEC, and that NEC does in fact occur in infants fed exclusively on human milk diets, a finding that, I think, fundamentally undermines the causal chain the lawsuits depend upon.

This scientific reality also illuminates a crucial practical truth: donor milk supply is chronically insufficient to meet neonatal intensive care unit (NICU) demand. Premature infant formula is in fact an essential medical intervention that sustains neurological and organ development when maternal milk is unavailable. Any attempt to litigate this product category out of existence may create a supply shortage in infant formula that could disrupt the operations of NICU .

Abbott itself has demonstrated remarkable conviction in this position. Rather than pursuing settlements that would implicitly concede fault, Abbott has taken a firm, principled stance: its products are backed by scientific research, and it will not capitulate to litigation pressure. The company has even expressed a willingness to divest the segment entirely before accepting unwarranted liability. Judgements to date have been mixed and remain under appeal, further reinforcing that this is far from a settled matter.

Investors should also note a matter of proportionality. Pediatric nutrition constitutes a relatively small portion of Abbott's total revenue. The litigation risk, while real in headline terms, does not threaten the structural foundation of the company. Abbott continues to deliver consistent growth in its medical devices segment and in its adult nutrition franchise. The litigation carries no contagion to these segments: cardiovascular devices, diabetes care, and diagnostics as they operate on entirely different commercial, regulatory, and liability frameworks.

Nevertheless, there are infants who die and, even though the medical community does not support the litigation, Abbott may still need to bear a significant amount of compensation that could put a toll on its financial statements given the mixed nature of judicial results.

II. The Exact Sciences Acquisition: EPS Dilution and Elevated Leverage

The second factor weighing on the share price is Abbott's acquisition of Exact Sciences, which has drawn criticism for EPS dilution and a meaningful increase in leverage. These concerns deserve to be weighed against the strategic logic, which is considerably more compelling than the near-term financial optics suggest.

Exact Sciences operates in oncology diagnostics, one of the highest-growth areas in modern medicine, and currently holds a position among the top three players in that space. The acquisition is expected to effectively double Abbott's addressable market in oncology diagnostics, a transformational expansion of total addressable market (TAM) that the current share price does not appear to price in. The business carries approximately 70% gross margins, a structurally attractive profile that would help bolster Abbott's beaten-down diagnostics segment following the normalisation of Covid-19 test demands where the operating margin has dropped under 20% from ~40% in 2022.

The synergy thesis is grounded in complementary capabilities. Abbott's distribution network spans over 160 countries, an asset that took decades and billions of dollars to build. Exact Sciences, by contrast, has been almost entirely U.S.-focused. The combination creates an immediate and credible international growth pathway for a diagnostics platform that, on its own, had no viable mechanism to replicate that global reach. The economies of scope across shared infrastructure, regulatory relationships, and commercial channels are genuine and quantifiable.

From a strategic timing perspective, this acquisition also addresses a real structural gap in Abbott's portfolio. Covid diagnostics represented an outsized revenue contributor that was going to normalise. Rather than simply accepting margin compression as that tailwind faded, Abbott has made a proactive move to replace that revenue with a durable, high-margin growth platform. That is capital allocation discipline, not recklessness.

On the balance sheet, the approximately \$20 billion in debt is indeed a valid concern. Abbott's debt-to-equity ratio has risen materially, to approximately 0.65x (as of June 2026) from 0.25x (2025) prior to the transaction. However, the critical mitigant is the structure of that obligation: the debt is pre-scheduled and spread across a 30-year repayment horizon, with predictable, fixed interest expenses that are entirely manageable against a business generating the cash flows Abbott produces. This is not a liquidity crisis or a covenant risk, it is a long-duration financing arrangement for a long-duration strategic asset. Additionally, thanks to years of capital discipline, Abbott has been operating on an extremely low financial leverage profile, which makes room for the current debt uptake required for the acquisition. Of course, we will need to keep a close eye on this to see whether the revenue increment from Exact Sciences can justify the elevated interest obligations and debt repayment.

Conclusion

The primary objective of writing this section is to express my thoughts on the potential reasons behind the struggling share price of Abbott. It is not to dismiss the risk but rather, I hope to provide a different perspective for investors to weigh their decisions. My core idea is that Abbott's business fundamentals have not changed where its medical devices, diagnostics equipment and established pharmacy still play a critical role in helping the public regain their health. These products are not easily substitutable given the psychological trust and an entire medical team having been trained on their products. The risk could bring short-term volatility to the stock price but, at the same time, it could present a rare opportunity for us.

4.0 Competitive Landscape

I understand that very few people have the time to wade through 100 to 200 pages of annual reports. To make these insights accessible, I used NotebookLM as an analytical co-pilot to help me distill the core essence of 3 major segments we have gone through previously: Diagnostics, Medical Device & Nutrition. In this section, I'm sharing those key takeaways so we can look past the raw numbers, understand the actual strategic narratives, and easily spot the nuances between industry rivals.

Diagnostics

In the diagnostics industry, value flows through a "hardware-to-annuity" cycle. The physical installation of complex instruments, such as Abbott's Alinity family or Danaher's Beckman Coulter platforms, serves as the initial hook. However, the real profit is captured downstream through proprietary reagents and consumables that must be purchased for the life of the machine, typically seven to ten years.

Management teams are increasingly using Total Laboratory Automation (TLA) as a structural lock-in. By installing a physical automation track like the GLP Systems Track, a company tethers the laboratory's entire workflow: sample movement, routing, and processing, to its specific ecosystem. This creates high switching costs, as replacing the system would require a massive operational overhaul and workforce retraining, effectively securing the reagent annuity against nimbler competitors.

The Narrative Reality

The diagnostic landscape is currently defined by a race to replace the "COVID cliff." For example, Abbott's COVID-testing sales fell from \$8.4 billion in 2022 to \$297 million in 2025. The reason behind recent shifts is a move from infectious disease testing (high velocity but temporary) toward chronic-care screening (steady and recurring).

- **Abbott's Pivot:** Management is trading pandemic-era liquidity for a permanent seat in oncology through the \$23 billion acquisition of Exact Sciences. They are betting that colorectal cancer screening (Cologuard) and breast cancer profiling (Oncotype DX) will provide the high-margin, recurring revenue needed to stabilize the segment.
- **The Scale Titans (Roche, Siemens, Danaher):** These players focus on "platform harmonization," trying to win large hospital contracts by offering a single, unified system for everything from clinical chemistry to hematology.
- **The Service Giants (Quest, LabCorp):** They compete by consolidating testing volume. Their strategy is inorganic growth, such as Quest's acquisition of LifeLabs, to achieve enough scale to dictate terms to smaller clinics and insurers.

Unmasked Assumptions

Management narratives rely on three critical qualitative judgments to justify their current valuations:

- **The Global Multiplier:** Management assumes that because they operate in over 160 countries, they can automatically scale U.S.-centric products like Cologuard globally. This ignores the different regulatory hurdles and reimbursement models in emerging markets.
- **Goodwill Stability:** Abbott recorded \$11.4 billion in goodwill for the oncology pivot Artifact 2, previous turned analysis. This assumes that future synergies and market growth in the \$60 billion oncology sector will never falter, avoiding a massive future write-down.
- **The "Base Business" Recovery:** Management frequently uses the phrase "organic growth in the base business" to divert attention from the fact that total segment revenue and margins have declined significantly since 2021 (dropping from 40.2% to 19.5%).

Contradictions & Blind Spots

The Margin Gap: While product-level benchmarks often suggest reagent margins of 60% or higher, Abbott's reported Diagnostic segment gross margin was approximately 44.2% in 2025. This suggests that the cost of maintaining hardware, providing services, and competing for contracts is significantly dragging down the theoretical profitability of the "blade" annuity.

China Pricing Pressures: Management notes "challenging market conditions" in China but provides little detail on the long-term damage from Volume-Based Procurement (VBP). These government tenders force massive price cuts that can permanently lower the profit floor in one of the world's fastest-growing markets.

Nutrition

Value in the nutrition industry is not captured through traditional retail marketing, but through two specific "lock-in" mechanisms:

- **Clinical Gatekeepers:** For brands like Ensure and Similac, value flows through a "scientific pull" strategy where physicians act as the primary distribution channel. By securing #1 doctor recommendations, companies create high switching costs for parents and patients who rely on clinical authority.
- **Institutional Monopolies (WIC):** In the U.S. pediatric market, the federal WIC program (Special Supplemental Nutrition Program for Women, Infants, and Children) acts as the industry's volume floor. Abbott currently holds exclusive contracts in 42 states, providing a guaranteed, multi-year distribution channel that competitors cannot penetrate until the contract expires.
- **The Rebate Gap:** A critical friction point is that between 48% and 49% of gross revenues are subject to rebates and allowances. This means nearly half of the headline sales figure is lost to institutional payers (GPOs and state agencies) before reaching net revenue.

The Narrative Reality

The shifts in this segment over the last three years are not driven by market expansion, but by a volumetric recovery cycle and aggressive pricing defense.

- **Market Share Seesaw:** Following Abbott's 2022 manufacturing stoppage at its Sturgis facility, Mead Johnson captured temporary market share. The 2023–2025 period represents a "rebasings" where Mead Johnson surrendered those gains as Abbott normalized its supply chain and returned to its market-leading position in the U.S. by Q4 2023.
- **Price over Volume:** Growth for both players has become heavily dependent on price increases rather than volume. In 2023, Abbott's 11.6% organic growth was almost entirely price-driven (+11.4% price vs. +0.2% volume). By 2024 and 2025, volume growth for Abbott turned negative (-1.7% and -1.3% respectively) as management prioritized margin preservation over absolute units.
- **Portfolio Pruning:** Abbott is moving to eliminate lower-margin "lifestyle" brands (e.g., the discontinuation of ZonePerfect in 2024) to focus manufacturing capacity on clinically-led brands like Ensure and Glucerna.

Unmasked Assumptions

Management narratives rely on three critical qualitative judgments to justify their performance:

- **Consumer Elasticity:** Abbott assumes it can continue to use "price adjustment initiatives" to offset permanent floors of higher manufacturing and input costs without permanently destroying volume.
- **The "Non-Core" Designation:** Mead Johnson's parent (Reckitt) has designated the business as "non-core," signaling a transition toward disposal. This assumes that a buyer will accept a business that recently suffered a massive supply chain shock (the 2024 Mount Vernon tornado) and is facing significant litigation.
- **Supply Chain Redundancy:** Abbott assumes that its new investments in "distributed manufacturing" (e.g., adding capacity in Ohio and Ireland) will provide a sufficient hedge against future regional facility stoppages or trade policy changes.

Contradictions & Blind Spots

The NEC "Black Box": There is a sharp disconnect between the front of the report and the legal footnotes regarding Necrotizing Enterocolitis (NEC) litigation. While management states they cannot "reasonably estimate a range of possible loss," juries have

already awarded damages of \$495 million and \$70 million against Abbott. The decision by courts that formula is "food" rather than a "medical product" removes the "learned intermediary" defense, placing the full liability for parent-level warnings on the manufacturer.

Volume Fragility: Management repeatedly highlights "market share recovery," but ignores the fact that total segment volume was negative in 2024 and 2025. This indicates that while they are recovering share from Mead Johnson, the total category demand may be shrinking or reaching a pricing ceiling.

WIC Concentration: While WIC contracts provide a volume floor, management does not provide a qualitative explanation for the risk of losing a large-state contract, which would create a volumetric hole that pricing increases could not easily fill.

Medical Device

Value in this segment flows through three primary pathways, each with different bargaining power dynamics:

- **The Institutional "Scientific Pull":** In high-acuity repairs (Structural Heart, Electrophysiology), clinicians act as the primary gatekeepers. Companies like Abbott and J&J utilize specialized clinical training as a moat; once a surgical team is proficient in a specific system like MitraClip or VARIPULSE, switching costs become prohibitive.
- **The Pharmacy Channel (Consumerized MedTech):** Dominated by the Diabetes segments of Abbott and Dexcom, value is captured through high-volume, recurring "razor-and-blade" sales. These firms increasingly bypass traditional hospital procurement to sell directly through retail pharmacies.
- **Group Purchasing Organizations (GPOs) & Tenders:** These entities negotiate supply contracts on behalf of hundreds of member hospitals. By concentrating purchasing decisions, they create a "narrow funnel" that forces manufacturers to compete in a transparent Dutch auction where price is often the only differentiator. Besides, there are national tenders (China VBP) where we can refer to the volume-based procurement (VBP) system in China. Under this model, the state-run tender effectively dictates the "net price" a company can charge across an entire province or nation in exchange for a guaranteed volume floor

Strategic Battlegrounds

Direct competitor overlap is most intense in four high-margin submarkets:

- **Diabetes (CGM): Abbott (Libre) vs. Dexcom (G7) vs. Medtronic (Guardian):**
Abbott: Global volume leader (~8M users) focusing on "Innovation for Access" and lowest list pricing. Dexcom: Pure-play focusing on MARD accuracy and rapid iteration. Medtronic: Focuses on "attachment rates" to its insulin pumps rather than standalone sensor volume.
- **Leadless Pacing: Abbott (Aveir) vs. Medtronic (Micra):** Abbott: Executed a "Tech Leapfrog" with the world's first dual-chamber leadless system, addressing a larger portion of the pacing market. Medtronic: Maintains pioneer status with the Micra platform, focusing on battery longevity.
- **EP (Pulsed Field Ablation): J&J (VARIPULSE) vs. Medtronic (PulseSelect) vs. Abbott (Volt):** J&J: Market leader with over 40,000 patients treated. Medtronic: Secured the first-ever FDA approval for PFA. Abbott: The Volt system aims to win on lab efficiency by integrating mapping and ablation into a single catheter.
- **Structural Heart: Abbott vs. Medtronic vs. Edwards:** Abbott: Dominant moat in mitral repair (MitraClip) and first-mover in tricuspid repair (TriClip). Medtronic: Strongest in the TAVR (aortic) market with the Evolut platform.

Financial Realities: The Efficiency Gap

A forensic look at the numbers reveals a massive divergence in how these companies convert innovation into shareholder value:

- **Abbott (The Efficiency Leader):** Boasts a segment operating margin of 33.7% in 2025, which has expanded annually through disciplined "bending of the cost curve" using AI and advanced manufacturing.
- **Johnson & Johnson (The M&A Gambler):** Operates with a depressed MedTech margin of ~12.2% because it is aggressively "buying its way" into leadership through multi-billion dollar acquisitions like Shockwave (\$13B) and Abiomed. J&J maintains the highest R&D intensity at ~18% of sales.
- **Medtronic (The Scale Titan):** While its reportable segment margins appear high (~35%), this excludes significant corporate and restructuring costs. Medtronic faces structural headwinds from China VBP and higher labor costs in hospital-based procedures.

Unmasked Assumptions & Structural Risks

Management teams across the sector are relying on three critical qualitative judgments:

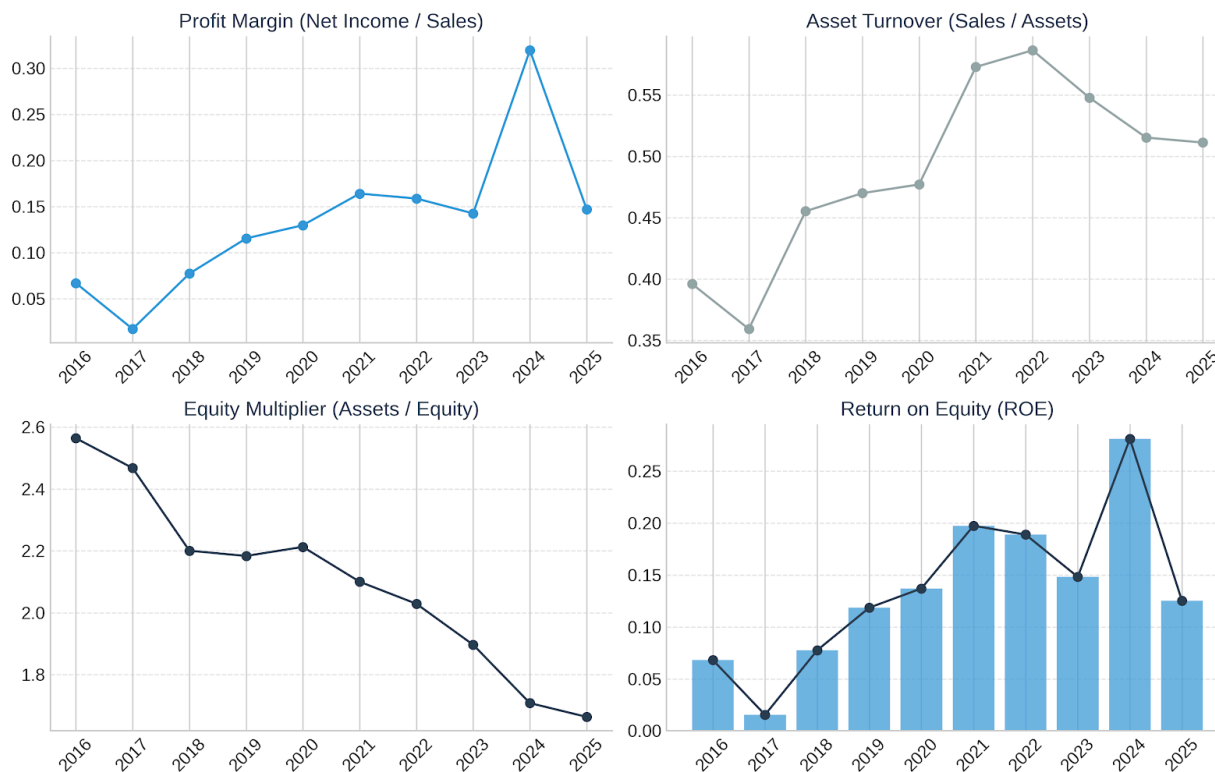
- **The "GLP-1 Companion" Narrative:** Both Abbott and Dexcom assume that GLP-1 weight-loss drugs are "companion products" that will drive CGM adoption rather than render it obsolete, a belief currently being tested by consumer demand shifts.
- **Medicare Pricing Floors:** Abbott and Dexcom face a looming "pricing cliff" on January 1, 2028, when Medicare begins competitive bidding for CGMs, which is expected to significantly decrease reimbursement rates.
- **China Vulnerability:** Rivals J&J and Medtronic are uniquely exposed to China's Volume-Based Procurement (VBP), which applies permanent downward pressure on pricing for staples, energy devices, and stents.

5.0 Financial Ratios

Profitability

	2021	2022	2023	2024	2025
Gross Margin	57.0%	56.2%	55.2%	55.4%	56.4%
EBIT Margin	19.6%	19.2%	16.2%	16.3%	18.2%
Net Profit Margin	16.4%	15.9%	14.3%	32.0%	14.7%
ROE	20.6%	19.1%	15.2%	31.1%	13.1%
ROA	9.6%	9.3%	7.8%	17.3%	7.8%
ROIC	16.4%	15.9%	12.4%	10.5%	11.2%

DuPont Analysis (Breakdown of ROE)



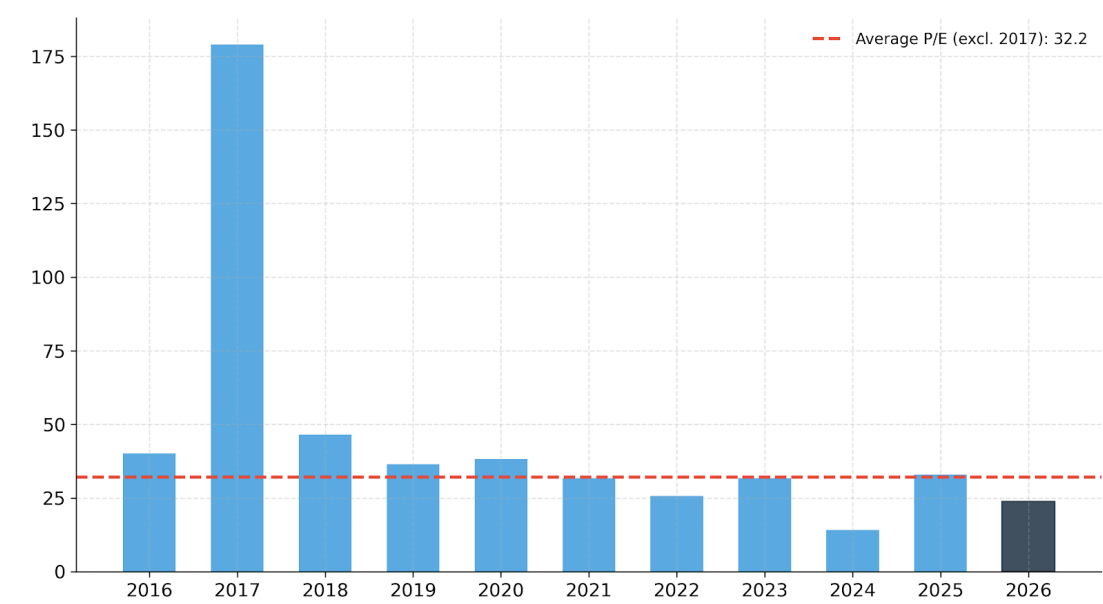
Liquidity and Solvency

	2021	2022	2023	2024	2025
Current Ratio	1.85x	1.63x	1.64x	1.67x	1.58x
Quick Ratio	1.46x	1.23x	1.16x	1.23x	1.18x
Debt / Equity	0.50x	0.46x	0.38x	0.30x	0.25x
Debt/Asset	0.24x	0.23x	0.20x	0.17x	0.15x
Interest Coverage	15.81x	14.99x	10.17x	12.21x	16.33x

**Debt includes only short-term and long-term debts*

Valuation

PE Ratio (Average PE excluded 2017 outlier)



Analysts’ Target Price (25 analysts)

High	Median	Low
\$ 143	\$ 115	\$ 92

Source: Capital IQ

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